

Chapter 8 (New): The Operations Pillar — Executing System Transformation

From Plans to Action: The 14-Hospital Process, RHRC Methodology, Workforce Architecture, and the Mechanics of Regionalization

Sources: Oliver Wyman Act 167 Community Engagement: Recommendations (2024); AHS Health Care System Transformation Reports (August 2025, November 2025); AHS January 2025 Legislative Presentation; Act 68 of 2025; Vermont RHT Program Application (November 2025); GMCB 2024 Annual Report; Vermont Blueprint for Health data.

The Operations Pillar: Where Strategy Meets Reality

Every chapter in this book that precedes this one is, in some sense, about ideas: the analytical framework for understanding Vermont's crisis, the financial architecture of global budgets and reference-based pricing, the clinical models that should replace hospital-centric care. This chapter is about execution. The Operations pillar is where the gap between a well-designed plan and an implemented one is either closed or exposed. It is where organizational inertia, workforce constraints, data gaps, political friction, and the sheer complexity of coordinating 14 independent hospital organizations across a geographically vast rural state determine whether transformation actually happens.

Vermont's operations challenge is unusual in its specificity and urgency. AHS has statutory deadlines — hospital transformation plans by early 2026, RBP by FY2027, global budgets by FY2028, the Statewide Strategic Plan by December 2028. It has a constrained budget (\$4.2M in appropriated state funds, plus \$195M in RHT Program capital and \$1.2M in AHEAD cooperative agreement funds). It has a newly contracted operational partner in the Rural Health Redesign Center. And it has 14 hospitals in wildly different financial and operational conditions, ranging from a large academic medical center at the center of the state's economy to 19-bed critical access hospitals in the Northeast Kingdom where any service reduction has immediate community consequences.

This chapter develops the operational framework for executing system transformation at the scale Vermont requires. It draws on the specific methodologies AHS and RHRC have deployed, the transformation planning process and its current state, and the lessons from other state and health system transformation efforts about what the operations discipline requires. It is written both as an academic treatment of the operations pillar and as a practical guide for the AHS staff, hospital leaders, and legislative overseers who are executing Vermont's transformation in real time.

Section 1: The Vermont Transformation Operating Model

Vermont's health system transformation is not being executed by a single authority acting on a blank canvas. It is being coordinated by AHS across a network of 14 independent (all non-profit) hospitals, 12 local health district offices, the GMCB as regulatory partner, the Department of Vermont Health Access (DVHA) as Medicaid authority, dozens of community-based organizations, primary care practices, designated mental health agencies, and a state legislature that requires monthly reporting on progress. Understanding the governance and operational model for managing this network is prerequisite to understanding how transformation actually works.

The Institutional Architecture

Act 68 established a four-actor governance architecture for Vermont's transformation that is worth understanding precisely:

Actor	Statutory Role	Key Authority	Current Capacity
-------	----------------	---------------	------------------

Agency of Human Services (AHS)	Lead transformation authority; develops and implements the Statewide Health Care Delivery Strategic Plan; provides technical assistance to hospitals	Directs transformation planning; awards transformation grants; sets SDOH investment priorities; manages RHRC and other contractors	Health Care Reform Office (recently reorganized); contracted RHRC through October 2025; hiring additional staff with RHT Program funds
Green Mountain Care Board (GMCB)	Regulatory authority; implements RBP and global budgets; reviews and approves hospital budgets; monitors quality and price	Sets hospital rates; approves/denies hospital budget requests; enforces Act 68 spending reduction requirements; issues CON decisions	Added 3 new permanent positions (Act 68 authorization); hired Affordability and Pricing Project Director; procuring RBP vendor (early 2026)
Department of Vermont Health Access (DVHA)	Medicaid authority; manages Medicaid global budget under AHEAD alignment; oversees Blueprint for Health and OneCare wind-down	Medicaid contract terms; Blueprint for Health funding; Medicaid global budget design under AHEAD	Managing AHEAD Medicaid alignment; Blueprint for Health expansion; post-OneCare transition
Health Care Delivery Advisory Committee	Provides accountability oversight; sets affordability benchmarks; advises on evaluation process for the health care delivery system	Advisory authority; stakeholder input; ongoing monitoring of strategic plan progress	Newly established under Act 68; initial composition and operating procedures being developed

Figure 8.1 — Vermont health system transformation governance architecture under Act 68. Sources: Act 68 of 2025; AHS Legislative Presentation (January 2025).

The governance architecture has one significant design tension worth naming: AHS is simultaneously the transformation planner, the technical assistance provider, the grant-maker, and the accountability monitor for the hospitals it is also trying to help. This is not an unusual arrangement in state healthcare reform — Maryland's HSCRC has regulatory authority and collaborative relationships simultaneously — but it requires explicit management. The AHS model works best when hospitals see the agency as a genuine partner in solving difficult problems, not primarily as a regulator imposing mandates. Oliver Wyman's recommendation to add a Division of Planning and Effectiveness and to clarify the AHS-GMCB role division is partly a response to this tension.

The Transformation Planning Sequence

AHS designed a phased transformation planning process that moves hospitals from initial engagement through draft strategy to final implementation plans. The sequence was developed with RHRC and reflects the operational reality that 14 hospitals in very different financial and operational positions cannot move at the same pace or toward the same model.

Phase	Content	Timing	AHS role
1. Data review and baseline	RHRC meets individually with each hospital to review hospital-specific financial and operational data: cost per adjusted discharge, operating margin, payer mix, service line volumes, workforce indicators, and	May–June 2025	RHRC provides technical assistance; AHS reviews data with hospitals; GMCB provides benchmark data

	comparison against national and Vermont peers		
2. Statewide and regional convenings	All 14 hospitals participate in statewide planning conversations; then organized into regional groups for more detailed discussion of shared opportunities: transfer coordination, shared clinical networks, service alignment, administrative consolidation	Summer–Fall 2025	AHS and Blueprint for Health convene; regional groups self-organize around specific opportunity areas
3. Draft Care Transformation Strategy Outline	Each hospital develops a written outline of its proposed transformation strategy, mapping short-term (within 1 year), medium-term (1-3 years), and long-term actions to Act 167 outcome objectives	Fall 2025	AHS reviews outlines; provides feedback; all eligible hospitals submitted applications for Act 68 transformation grants
4. Draft Transformation Plan	Full transformation plan with specific action steps, associated costs, timelines, service line analysis, staffing models, and shared service opportunities. Aligned with regional and statewide planning.	Winter 2025–2026	AHS reviews and approves each draft; GMCB provides analytical support; analytics vendor (being procured) enables cost modeling
5. Final Transformation Plan	Approved final plan; AHS integrates hospital plans into statewide picture; plans inform AHS's Statewide Strategic Plan development for December 2028 delivery	Early 2026 and ongoing	AHS integrates across 14 plans; identifies statewide priorities; allocates RHT Program capital to support implementation

Figure 8.2 — Vermont hospital transformation planning sequence. Sources: AHS Health Care Transformation website; AHS November 2025 Act 68 Report; AHS January 2025 Legislative Presentation.

"Transformation work is ongoing and evolving. Based on hospital feedback and changes in state and federal policy, AHS is continuing to adapt our approach to ensure planning is coordinated, responsive, and effective."

— Vermont Agency of Human Services, Health Care Transformation website, 2026

Section 2: The RHRC Methodology — Technical Assistance for Rural Hospital Transformation

The Rural Health Redesign Center (RHRC) was AHS's primary operational contractor for hospital transformation planning from spring 2025 through October 2025. Understanding the RHRC methodology is important not just as Vermont history but as a transferable model for how state agencies can provide technical assistance to financially distressed rural hospital networks — a challenge that will confront state health agencies across the country as the demographic and financial pressures Vermont faces today spread nationally.

What RHRC Did

RHRC's engagement combined three functions that are usually performed separately: financial and operational assessment, change management support, and transformation

plan development. The combination was deliberate — Oliver Wyman's experience had shown that hospitals can absorb financial analysis without changing behavior, can engage in planning processes that produce documents rather than action, and can participate in change management programs without addressing their fundamental financial unsustainability. RHRC's model integrated all three.

RHRC's Four Workstreams at Each Hospital

RHRC worked with each of Vermont's 14 hospitals through four parallel workstreams, adapted to each hospital's specific position and needs:

- Financial and operational baseline: Review of hospital-specific cost data from GMCB financial records, CMS cost reports, and the NASHP Hospital Cost Tool. Identification of the hospital's position relative to Vermont average and national benchmark on key efficiency metrics (operating profit per adjusted discharge, management and administrative cost per adjusted discharge, direct patient care labor cost per adjusted discharge).
 - Priority identification: Working with hospital leadership to identify the highest-leverage short-term cost reduction opportunities — supply chain, staffing models, service line analysis — and medium-term transformation opportunities — shared services, service line reconfiguration, COE alignment.
 - Transformation plan development: Supporting each hospital in developing a structured transformation plan that maps specific actions to timelines, costs, and Act 167 outcome objectives. Plans address hospital sustainability and cost through analysis of service lines, staffing models, and shared services opportunities.
 - Regional coordination: Connecting hospital-level planning to the statewide regional work — ensuring individual hospital plans are aware of and compatible with what neighboring hospitals are planning, and identifying shared service opportunities that require cooperation across organizational boundaries.
-

The RHRC contract concluded in October 2025 — a planned conclusion, not a termination. AHS designed the engagement as a time-limited capacity-building investment, not an ongoing dependency. The explicit goal was for hospitals to develop the internal planning capability and the collaborative relationships needed to continue transformation work independently, with AHS providing ongoing oversight, grant funding, and analytical support rather than direct technical assistance.

This design philosophy reflects a lesson from failed state transformation initiatives: external consultant-driven processes that do not build internal organizational capacity produce plans that expire when the consultant leaves. Vermont's approach — using RHRC to build the planning process and the inter-hospital relationships, then transitioning to AHS-led coordination with hospital-level implementation — is more likely to produce durable change.

The Analytics Gap and Its Resolution

The November 2025 AHS transformation report identified one critical operational gap in the current system: the absence of an analytics platform capable of evaluating hospital transformation proposals quantitatively, modeling the cost and quality implications of different service configurations, and assessing alignment with statewide goals. This gap means AHS currently cannot answer with confidence the questions that the transformation planning process is supposed to produce answers to: if Springfield Hospital converts to a Rural Emergency Hospital, what is the net financial impact on the system? If orthopedic surgery is concentrated at Southwestern Vermont and Copley, what travel burden does that create for patients in the affected communities?

AHS responded to this gap by procuring an analytics vendor in partnership with GMCB — a joint procurement that reflects the appropriate division of labor: GMCB brings regulatory data and rate-setting expertise; AHS brings population health and SDOH context. The analytics platform, once operational, will enable the state to evaluate hospital proposals in terms of quality, cost, and access; assess alignment with regional and statewide goals; and quantify potential cost savings using evidence-based

methods. This platform is also the technological foundation for monitoring the Statewide Strategic Plan's implementation metrics after December 2028.

Section 3: The Vermont Hospital Performance Dashboard — Where Each Hospital Stands

AHS's November 2025 transformation report established Vermont's official performance measurement framework — the set of outcome measures against which progress toward Act 167's five goals will be tracked. The framework covers three domains: provider efficiency and sustainability, payer affordability, and patient health outcomes. The following table presents the current performance landscape, with Vermont's actual values compared to benchmarks set at the 75th percentile of national peers.

Measure	VT Current	Benchmark (75th pctile)	Gap	Act 167 Goal
30-Day all-cause readmission rate	14.8%	14.5%	+0.3% (within range)	Improve health outcomes
Potentially avoidable ED visits as % of all ED visits	32.3%	No benchmark set yet	Significant opportunity	Reduce inefficiencies; access
Operating profit per adjusted discharge — PPS hospitals	\$5,345	\$4,162	+\$1,183 above benchmark	Lower costs
Operating profit per adjusted discharge — CAH hospitals	\$938	\$2,784	-\$1,846 BELOW benchmark	Lower costs / sustainability
Mgmt. and admin cost per adjusted discharge — PPS	\$2,730	\$1,427	+\$1,303 above benchmark	Reduce inefficiencies
Vermonters with a personal health care provider (PCP)	91%	87%	+4% above benchmark	Access / equity
Primary care practices accepting new Medicaid patients	59%	Not set	Access gap for Medicaid	Access / equity
Blueprint PCMH patients with hypertension controlled	77%	Not set	Quality management	Improve health outcomes
ED visits for suicide ideation/self-harm per 10K ED visits	245.5	Not set	Behavioral health access gap	Access / equity
ED visits for opioid overdose per 10K ED visits	16.1	Not set	SUD treatment access gap	Access / equity

Figure 8.3 — Vermont health system transformation outcome measures: current performance vs. benchmarks. Source: AHS Health Care System Transformation Report, November 2025.

Several findings from this dashboard deserve specific attention from healthcare leaders and policymakers:

The CAH operating profit gap is the most urgent financial signal. Critical Access Hospitals are running at \$938 operating profit per adjusted discharge, against a benchmark of \$2,784 — a \$1,846 per-discharge gap. This means Vermont's smallest, most rural hospitals are the furthest from financial sustainability. The RBP and global

budget reforms will affect these hospitals differently from PPS hospitals, and Act 68 explicitly requires GMCB to consider CAH-specific adjustments. The RHT Program's explicit inclusion of CAH stabilization as a priority use of funds reflects this gap.

The administrative cost premium is massive. PPS hospitals are spending \$2,730 per adjusted discharge on management and administrative costs, against a benchmark of \$1,427 — a 91% premium over what efficient peer hospitals spend. This \$1,303 per-discharge gap is exactly the administrative cost reduction that Oliver Wyman identified as one of the three primary sources of direct savings from transformation. If Vermont's PPS hospitals can reach the benchmark administrative cost level, the system savings are substantial — and Oliver Wyman estimated administrative reduction as contributing over \$100 million of the \$400M+ transformation savings target.

The behavioral health gap is measurable and alarming. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — and rates statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — the crisis presentation data confirms what Oliver Wyman's community meetings documented: Vermont's behavioral health access problem is driving a significant share of avoidable hospital utilization. This is not a clinical quality problem; it is an access problem. The CCBHC expansion (5 new entities by July 2026) and the behavioral health investments funded through AHEAD's EAST Fund are the operational response.

Management and administrative cost per adjusted discharge: Vermont PPS hospitals vs. benchmark

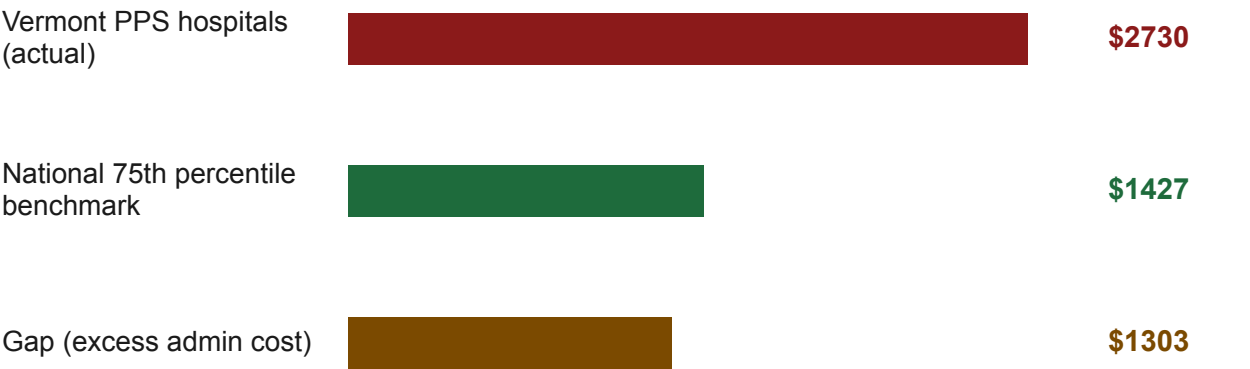


Figure 8.4 — Administrative cost gap: Vermont PPS hospitals vs. national benchmark. This \$1,303 per-discharge gap represents the primary operational inefficiency target for Vermont's transformation. Source: AHS Transformation Report, November 2025 (NASHP via CMS).

Section 4: Regionalization — The Operational Logic of Right-Sizing a 14-Hospital System

Regionalization is the operational translation of Oliver Wyman's third imperative: move all care possible out of hospitals, and concentrate what must stay in hospitals at sites with sufficient volume to maintain expertise. The concept is straightforward; the execution is complex. This section develops the operational methodology of regionalization — how you actually move from a system of 14 hospitals each attempting full-spectrum care to a differentiated regional network where each facility has a clearly defined role, adequate patient volume, and sustainable financial structure.

The Regional Network Design Framework

Oliver Wyman's regionalization blueprint organized Vermont's hospital network around three facility types, each with a distinct role in the transformed system:

Tier 1 — Regional Specialty Centers (RSCs)

RSCs are the hospitals with sufficient population base, financial position, and existing expertise to sustain inpatient beds and serve as Centers of Excellence in multiple specialty areas. Under Oliver Wyman's blueprint, UVMMC in Burlington is the state's primary RSC with 16 COE designations. Southwestern Vermont Medical Center, Brattleboro Memorial Hospital, Rutland Regional Medical Center, and Central Vermont Medical Center function as secondary RSCs, each with 5-9 COE designations serving their regional populations.

The RSC model requires a fundamental change in how these hospitals think about their role: from serving their immediate geographic catchment area to accepting referrals from the entire region for their designated specialties. This means building the transfer infrastructure, the outreach relationships with smaller hospitals, and the care management capacity to serve a larger and more complex patient population.

Tier 2 — Community Hospitals with Focused Scope

Community hospitals with focused scope are inpatient facilities that maintain specific service lines where they have adequate volume and expertise — their designated COE specialties — while transferring other complex cases to RSCs. Northwestern Medical Center (St. Albans), Northeastern Vermont Regional Hospital (St. Johnsbury), Springfield Hospital, and Copley Hospital (Morrisville) fall into this category, with 2-4 COE designations each.

The operational challenge for this tier is managing the transition: reducing reliance on service lines being regionalized elsewhere while building capacity in designated specialties and managing the patient volume shift without destabilizing the hospital's financial position. This requires precisely the kind of short-term, medium-term, and long-term planning that RHRC's transformation plan methodology was designed to support.

Tier 3 — Transformed Facilities (REH, CACC, or Home-Based Models)

Tier 3 facilities are hospitals that cannot sustain inpatient beds in the long term — either because their population base is too small, their financial position is too distressed, or both. Rather than closing unexpectedly and leaving communities without any healthcare access, Oliver Wyman's blueprint proposes orderly conversion to one of three alternative models: Rural Emergency Hospital (REH — 24/7 emergency and observation without inpatient beds), Community Ambulatory Care Center (CACC — outpatient and primary care services), or a Care at Home support hub. Grace Cottage,

Gifford Medical Center, North Country Hospital, and Porter Medical Center are among the facilities whose futures require further discussion as part of Vermont's regionalization planning.

The REH designation is particularly significant for Vermont because it preserves the emergency access function that communities depend on while eliminating the expensive inpatient infrastructure that these hospitals cannot financially sustain. A community with an REH and reliable EMS transport to an RSC 45 minutes away is better served than one with a financially distressed inpatient hospital that cannot staff its surgical suite or maintain specialist coverage.

Hospital (HSA)	Curr ent COE s	Tier	Key transformation considerations
UVM Medical Center (Burlington)	16	Tier 1 — Primary RSC	Administrative overhead reduction is critical — \$3,826 admin cost/adj. discharge vs. \$1,427 benchmark. Examine non-patient-care provider activities (OW specific finding).
SW VT Medical Center (Bennington)	9	Tier 1 — Secondary RSC	Strong COE position. Key southern VT anchor. Psych adult/adolescent COE important given behavioral health gaps.
Brattleboro Memorial Hospital	6	Tier 1 — Secondary RSC	Acute general surgery COE is regionally critical. Windham County has higher-than-average suicide/SH and opioid rates — behavioral health access priority.
Rutland Regional Medical Center	5	Tier 1 — Secondary RSC	Central Vermont anchor; neurology and psych-adult COEs important. Rutland County has higher opioid overdose rates.
Central VT Medical Center (Barre)	5	Tier 1 — Secondary RSC	Radiation therapy and neurology COEs. Washington County (Barre/Montpelier) is second most populous HSA.
NW Medical Center (St. Albans)	4	Tier 2 — Focused scope	Northern VT anchor. Cancer surgery and radiation COEs. Franklin County workforce hub.
NE VT Regional Hospital (St. Johnsbury)	4	Tier 2 — Focused scope	Northeast Kingdom anchor — Essex/Caledonia/Orleans have highest uninsurance rates (6-8%) and most rural populations.
Springfield Hospital	2	Tier 2 — Focused scope	Memory care and psych-adult COEs. High SUD rates in Windsor/Windham corridor.
Copley Hospital (Morrisville)	2	Tier 2 — Focused scope	Orthopedics and rheumatology COEs. Lamoille County access point. Rural workforce housing challenge.
Mt. Ascutney (White River Jct.)	1	Tier 2 — Focused scope	Rehabilitation COE. Dartmouth Health affiliation creates cross-border care opportunities.
Grace Cottage, Gifford, North Country, Porter	TBD	Tier 3 — Requires discussion	Further assessment needed. REH conversion, CACC, and home-based care models all under consideration. RHT Program funds available for transition support.

Figure 8.5 — Vermont 14-hospital regionalization framework: COE designations, tiers, and key operational considerations. Sources: Oliver Wyman Act 167 Report (2024); AHS Transformation Reports (2025).

The Operational Mechanics of Regionalization

Regionalization is not a policy decision that happens once — it is an operational process that must be managed over years. The transition from a hospital system where every facility attempts full-spectrum care to one organized around regional specialization requires changes across five operational domains:

Transfer infrastructure is the most visible immediate need. Oliver Wyman documented that Vermont currently has 31 separate EMS agencies that a provider may need to contact to arrange a patient transfer — a fragmentation that is simultaneously a patient safety risk and an operational cost. Vermont's RHT Program application includes EMS transformation as a priority investment: professionalization and regionalization of EMS, improved inter-facility transfer coordination, and expanded telehealth to reduce unnecessary transfers. The November 2025 AHS report identified inter-facility transfer coordination as one of the primary statewide regionalization themes emerging from regional hospital convenings.

Clinical network agreements formalize the referral relationships between RSCs and Tier 2/3 facilities. Oliver Wyman recommended that RSCs establish formal agreements to receive referrals from emergency departments at smaller hospitals for their designated COE specialties — ensuring that when a patient arrives at Springfield Hospital needing complex general surgery, the transfer pathway to Brattleboro Memorial Hospital (the acute general surgery COE for that region) is established, protocolized, and reliable. These agreements do not currently exist in a systematic form across Vermont's hospital network; the Act 68 transformation planning process is the vehicle for creating them.

Administrative shared services offer some of the largest near-term cost savings in the transformation agenda. Oliver Wyman identified three shared service opportunity areas as high-priority: supply chain (group purchasing), administrative functions (billing, coding, human resources, credentialing), and IT infrastructure. Vermont hospitals have begun early-stage discussions on group purchasing during the regional convenings. The administrative cost gap — \$1,303 per discharge above benchmark — is where shared services can make the most immediate impact on the \$400M+ savings target.

Workforce redistribution is the most sensitive operational challenge. When a hospital transitions from full inpatient to REH or CACC status, the clinical staff whose roles were tied to inpatient services must either transition to new roles, relocate to RSCs, or leave the healthcare workforce. Oliver Wyman explicitly recommended that healthcare workforce affected by system changes could be redistributed or retrained to perform services needed by the community — recognizing that the workforce challenge is not just about attracting new providers to Vermont but about optimally deploying those already there. Vermont's RHT Program workforce investments — tuition assistance, recruitment incentives with 5-year service obligations, scope-of-practice expansion for nurses and APRNs — are the supply-side response; workforce redistribution planning is the demand-side response.

Community engagement is the political prerequisite for all operational change. Oliver Wyman's process demonstrated that communities are willing to accept difficult changes — service reductions, facility reconfiguration, changes in travel distances — when they have been genuinely engaged in the decision-making process and when they understand the alternative (unexpected closures) is worse. AHS has committed to continuing community engagement as transformation plans develop, expanding beyond hospital leadership to include community providers and other health and human services stakeholders as priorities are identified.

Section 5: The Workforce Crisis — Vermont's Most Binding Operational Constraint

Of all the operational challenges Vermont faces, workforce is the one most likely to constrain the pace and ambition of transformation. Vermont's healthcare workforce problem is not a single issue — it is a cluster of interconnected problems that Oliver Wyman's systems map showed cascading across the entire healthcare system.

Understanding the workforce problem in its full complexity is prerequisite to designing interventions that actually work.

The Dimensions of Vermont's Workforce Crisis

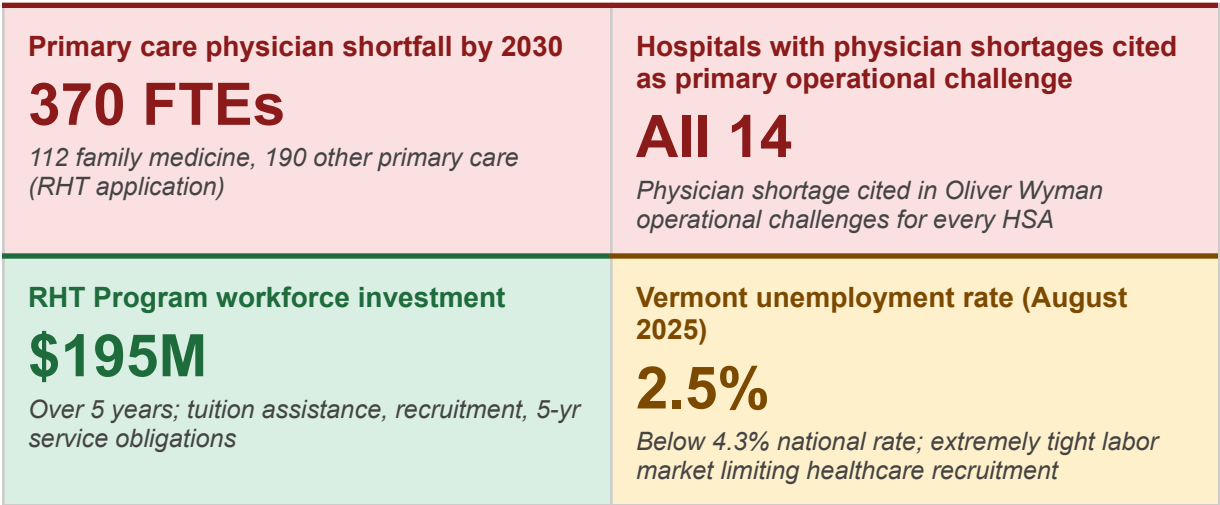


Figure 8.6 — Vermont workforce crisis key metrics. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Labor.

Vermont's workforce problem has five distinct but interconnected dimensions, each requiring a different intervention approach:

Primary care shortage is both a supply problem and a productivity problem. Oliver Wyman made a counterintuitive finding that Vermont's primary care shortage is partly a productivity issue, not purely a headcount problem: HRSA recognizes no Health Profession Shortage Areas in Vermont, and if primary care providers were supported to see three patients per hour, the state would have adequate supply. The implication is that scope-of-practice expansion for APRNs and physician assistants, team-based care models that use clinical staff at the top of their licensure, and administrative burden reduction (prior authorization reform, documentation support) can meaningfully increase primary care capacity without adding headcount.

Specialist shortage drives regionalization urgency. Vermont cannot maintain specialist coverage at 14 hospitals across a state of 647,000 people. The population density and economics do not support it. The Oliver Wyman COE framework is, in part, a response to the specialist shortage: rather than trying to recruit and retain a cardiologist or neurosurgeon for every Vermont community, concentrate those specialists at facilities with sufficient volume to justify their positions and deliver the clinical excellence their presence enables.

Nursing shortage is the most acute staffing crisis at individual hospitals. All Vermont hospitals cited physician shortages and difficulty recruiting staff since COVID-19 as primary operational challenges in Oliver Wyman's assessment. Travel nursing — the expensive emergency staffing solution that many hospitals have relied on since 2020 — has significantly worsened hospital financial positions. Travel nurse costs are typically two to three times the cost of a permanent hire. Vermont hospitals that remain heavily dependent on travel nursing are in a financial hole that deepens with every shift filled by a travel agency.

Housing is the hidden workforce constraint. Oliver Wyman's systems map traced the connection from housing shortage to workforce shortage to healthcare system fragility explicitly. Vermont's rental vacancy rate of 3% — well below the 5% rate of a healthy market — means healthcare workers recruited to Vermont communities cannot find housing. Half of Vermont renters are cost-burdened. This is not a healthcare problem that healthcare reform can solve; it requires housing policy responses. But AHS's transformation mandate, properly understood, extends to advocating for and enabling the housing investments that make healthcare workforce recruitment possible. This is one of the reasons Oliver Wyman's first imperative — build housing and fix transportation — comes before payment reform in the three-imperative sequence.

Scope-of-practice restrictions limit Vermont's ability to use the workforce it has effectively. Vermont has taken some steps toward scope-of-practice expansion — joining the Social Work Licensure Compact, allowing pharmacist-extended prescriptions, approving mental health professionals without master's degrees for certain psychotherapy roles — but Oliver Wyman's action list called for further expansion of professional licensure and practice scope for nurses, EMTs, and pharmacists. The RHT Program's workforce investments include scope-of-practice-enabling training, and Act 68's workforce goal — attracting and retaining high-quality health care professionals — implicitly requires the scope-of-practice environment to make Vermont competitive with surrounding states.

Vermont's Workforce Strategy: The RHT Investment Framework

Vermont's Rural Health Transformation Program application dedicated a substantial portion of its \$195M award to workforce infrastructure — treating workforce as capital investment rather than operating expense. The distinction matters: using one-time capital funds to build durable workforce pipelines (tuition assistance that creates committed local providers, training infrastructure that enables scope expansion, housing investments that make recruitment viable) is fundamentally different from using them to fund ongoing salaries.

RHT Workforce Investment Priorities

- Vermont's RHT Program application included five workforce investment areas, each designed to produce durable capacity rather than temporary staffing:
- Tuition assistance and loan forgiveness for nurses, physicians, APRNs, LNAs, home health aides, and other providers, with 5-year service obligations in Vermont — building a locally-rooted workforce pipeline
 - On-site clinical training infrastructure — supporting teaching programs that keep medical students and residents in Vermont communities during training, increasing the probability they establish practices there
 - Scope-of-practice training and support for APRNs, physician assistants, pharmacists, and EMTs — enabling Vermont to use its existing workforce at maximum capacity
 - Community health worker training and deployment — building the SDOH navigation and care coordination workforce that population health management requires, drawing from the communities being served
 - Interstate Medical Licensure Compact (IMLC) participation — enabling out-of-state providers to practice via telehealth in Vermont, reducing specialist access gaps without requiring physical relocation

Section 6: The Operations Metrics Framework — What AHS Must Track

Effective operations management requires clear, measurable indicators of progress. AHS's current transformation outcome framework (Figure 8.3) establishes a baseline measurement architecture, but it has acknowledged limitations: most data reflects 2023 or earlier performance, the impact of 2025 transformation activities will not appear in measures until 2026 or later, and some critical domains — mental health, SDOH, post-acute care — lack the consistent data infrastructure to support real-time monitoring.

For the Statewide Strategic Plan that AHS must deliver by December 2028, a more complete operations metrics framework is required. Drawing on Oliver Wyman's five Act 167 goals and Act 68's accountability metrics requirements, the following framework organizes the key operational indicators by domain:

Domain	Key metrics	Data source	Target direction
--------	-------------	-------------	------------------

Hospital financial sustainability	Operating profit per adjusted discharge by hospital type; management/admin cost per adjusted discharge; days cash on hand; bond ratings; travel nurse cost as % of total labor	GMCB hospital budget data; CMS cost reports; NASHP Hospital Cost Tool	CAH: toward \$2,784 benchmark. PPS admin cost: toward \$1,427 benchmark. Travel nursing: reduce toward 5-10% of total labor
Care access	% primary care practices accepting new Medicaid patients; wait times for new patient appointments; potentially avoidable ED visit rate; 30-day follow-up after ED visit for mental illness and SUD	VCCI; GMCB; VDH; Blueprint for Health	Primary care acceptance: increase from 59%. Avoidable ED visits: reduce from 32.3%. 30-day MH follow-up: increase from 76%
Population health	Hypertension control rate in PCMH patients; diabetes HbA1c not in control; all-cause readmission rate; ED visits for suicide ideation/self-harm; ED visits for opioid overdose	Blueprint for Health; VDH; CMS	Hypertension control: maintain 77%; diabetes: reduce 22% not-controlled; readmission: maintain at/below 14.8%
Cost and affordability	Per-member-per-month total cost of care trend; commercial premium growth rate; individual market affordability for medium-high utilization family; medical loss ratio for marketplace plans	GMCB; insurance rate filings; CMS; VHCURES	PMPM cost: below AHEAD total cost of care targets. Premium growth: reduce toward CPI. MLR: maintain 89%+
Equity	Geographic variation in primary care access by county; health outcomes variation by income and race/ethnicity; uninsured rate by county; Medicaid acceptance rates by region	VDH BRFSS; GMCB; VHCURES	Northeast Kingdom uninsurance gap: reduce from 6-8% toward statewide 3%. Medicaid acceptance: increase across all regions
Operations efficiency	Hospital-to-hospital transfer completion rate and time; EMS response time by county; shared service adoption rate; administrative cost trend; COE referral volume by specialty	EMS records; GMCB; hospital transformation plans	Transfer completion: establish baseline and reduce time. Admin cost trend: negative year-over-year. COE referral volume: increase as regionalization advances

Figure 8.7 — Vermont health system transformation: comprehensive operations metrics framework. Sources: Act 68 of 2025; AHS Transformation Report (November 2025); Oliver Wyman Act 167 Report.

One structural observation about this metrics framework: many of the most important indicators will not show measurable improvement for 18-24 months after the interventions that should drive them are implemented. Healthcare system change is slow, and the data infrastructure to measure it is even slower. AHS's acknowledgment in its November 2025 report that most available measures reflect 2023 or earlier performance is not an evasion — it is an accurate description of the measurement reality. Policymakers and oversight bodies need to understand this lag and resist the temptation to declare failure when early data does not yet show improvement.

Section 7: AHS as System Operator — Organizational Design for the 2028 Mandate

The operations chapter concludes with the organizational design question that underpins everything else: is AHS currently structured to execute the transformation it has been mandated to lead? The honest answer, reflected in Oliver Wyman's findings and AHS's own public reports, is: not yet, but the organization is restructuring.

The Current Organizational Gaps

Oliver Wyman identified four specific organizational gaps in AHS that impede transformation execution. These are not criticisms of individuals — they reflect an organizational structure designed for a different era that has not yet adapted to the system operator role Act 68 requires.

- **Misalignment between AHS sub-unit organization and Hospital Service Areas.** Vermont's AHS is organized around program types (mental health, substance use, aging services, Medicaid, etc.) rather than geographic service areas. This means that when a hospital in the Northeast Kingdom is trying to address the housing, transportation, behavioral health, and primary care gaps that drive its ED volume, it must work with multiple separate AHS programs — each with its own administrative requirements, reporting cycles, and performance metrics. Oliver Wyman's recommendation to align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA is a structural reorganization, not just a coordination improvement.
- **Absence of a Planning and Effectiveness function.** AHS currently lacks a dedicated analytical unit that can calculate the financial and access impacts of service site changes, monitor access and affordability across community providers, and track the quality, access, and equity metrics that the Statewide Strategic Plan will require. The analytics vendor procurement in partnership with GMCB addresses part of this gap, but the internal capacity to interpret, apply, and act on analytical outputs is a separate organizational requirement.
- **Under-resourcing for the transformation management function.** Act 68 requires AHS to simultaneously negotiate with CMS on AHEAD implementation, manage 14 hospital transformation planning processes, develop the Statewide Strategic Plan, oversee RHT Program implementation, coordinate with GMCB on RBP and global budget design, and report monthly to the legislature. The Health Care Reform Office, recently reorganized and supplemented by contractor support, is managing a workload that probably requires 50-70% more staff than currently exist. The RHT Program award explicitly includes funds for hiring staff to oversee its implementation.
- **Limited SDOH integration across AHS programs.** The Oliver Wyman analysis demonstrated that housing, transportation, food security, and behavioral health — all areas where AHS has programmatic authority — are primary drivers of healthcare utilization. But AHS's current organizational structure treats these as separate program areas rather than as integrated components of a population health strategy. The Statewide Strategic Plan mandate creates the opportunity to redesign this integration explicitly.

The Emerging Solution: A Population Health Management Operating Model

The organizational model that AHS's Strategic Plan should articulate — and that the AHS restructuring chapter later in this book develops in full — is a population health management operating model organized around Vermont's Hospital Service Areas, with clear roles for each level of the system.

At the statewide level, AHS functions as the system architect: setting the strategic direction, establishing the payment reform framework (in partnership with GMCB), allocating capital (RHT Program, AHEAD EAST Fund, Act 68 grants), and monitoring system performance against the five Act 167 goals.

At the HSA level, regional care networks — organized around the Tier 1 RSC for each region — function as the operational unit: coordinating transfer pathways, managing shared service arrangements, aligning primary care and behavioral health investment with hospital transformation plans, and connecting healthcare planning with housing, transportation, and SDOH infrastructure planning.

At the provider level, individual hospitals, primary care practices, designated agencies, FQHCs, and community-based organizations function as service delivery nodes: executing transformation plans, participating in regional coordination, and reporting performance against standardized metrics that allow AHS to monitor progress and identify where additional support is needed.

This three-level operating model is not new to health system design — it is the architecture of every effective regional health system in the country. What is new in Vermont is the explicit statutory mandate (Act 68's Strategic Plan requirement), the financial resources to build the infrastructure (RHT Program, AHEAD EAST Fund), and the political will — demonstrated through the extraordinary legislative sequence of Acts 167, 51, 55, and 68 — to execute the structural change rather than continuing to manage the decline.

Key Concepts Introduced in This Chapter

Term	Definition
Regional Specialty Center (RSC)	A hospital designated to concentrate volume and clinical expertise in multiple specialty areas, serving as the referral destination for those specialties across a multi-HSA region. Vermont's Tier 1 hospitals under the Oliver Wyman regionalization blueprint.
Rural Emergency Hospital (REH)	A federal CMS designation allowing a hospital to provide 24/7 emergency and observation services without inpatient beds. A financially sustainable alternative for Vermont hospitals that cannot sustain full inpatient capacity but must maintain emergency access for their communities.
Community Ambulatory Care Center (CACC)	An outpatient and primary care facility model proposed by Oliver Wyman as a potential conversion option for hospitals unable to sustain inpatient operations.
Potentially Avoidable ED Visit	An emergency department visit that could have been prevented through timely primary care, care coordination, or SDOH intervention. Vermont's rate of 32.3% of all ED visits represents a major quality and cost improvement opportunity.
Rural Health Redesign Center (RHRC)	AHS's primary technical assistance contractor for hospital transformation planning in 2025, providing financial assessment, change management support, and transformation plan development across all 14 Vermont hospitals. Contract concluded October 2025.
Hospital Service Area (HSA)	The geographic area defined around each Vermont hospital and the population it primarily serves. Vermont has 14 HSAs. Oliver Wyman recommended organizing AHS sub-units around HSAs to improve coordination between AHS programs and healthcare providers.
Travel nursing cost	The cost of temporary nurses hired through staffing agencies to fill vacancies, typically 2-3x the cost of permanent hires. Heavy travel nursing dependence is a primary driver of hospital financial distress across Vermont's system.
Administrative shared services	Centralized provision of non-clinical administrative functions (billing, coding, HR, credentialing, IT, group purchasing)

	across multiple hospitals, reducing the per-hospital administrative cost that is currently 91% above benchmark for Vermont PPS hospitals.
Population health management operating model	An organizational structure that manages healthcare resources at the population level — tracking health outcomes, utilization patterns, and SDOH factors across a defined geography — rather than managing individual patient encounters. The organizational model appropriate for a global budget environment.

Sources: Oliver Wyman Healthcare and Life Sciences Group, *Act 167 Community Engagement: Recommendations* (August 2024, revised October 2024); Vermont Agency of Human Services, *Health Care System Transformation Report* (November 1, 2025); Vermont Agency of Human Services, *Health Care System Transformation Report* (August 2025); Vermont Agency of Human Services / Brendan Krause, *Vermont's Health Care Reform Efforts*, House Health Care Committee (January 31, 2025); Act 68 of 2025 (Vermont); Vermont Rural Health Transformation Program Application (November 3, 2025); Green Mountain Care Board, *2024 Annual Report* (January 15, 2025); Vermont Agency of Human Services, *Health Care Transformation website* (healthcarereform.vermont.gov), accessed April 2026.